Table 10. Description of the comparative clinical study

Study ID	No. of Study Centres Locations	Study Start Completion Date Total Enrolment/ Enrolment Goal	Study Design Control Type	Study and Control Drugs, Dose, Route, and Regimen[a]	Study Objectives	No. Subjects by Group Entered/ Completed	Study Duration	No. M/F Median Age (Range)	Key Inclusion Criteria	Efficacy Endpoints
Study Reports of Controlled Clinical Studies Pertinent to the Claimed Indications										
Study 20190232	84 Poland, Germany, Estonia, Latvia, Lithuania, Hungary, Canada, and the United States	11 November 2020 (start) 03 June 2022 (completion) 563/542	Randomised, multicentre, double-blind, active-controlled, multiple-dose study	ABP 654 or ustekinumab (EU); 45 mg (baseline BW ≤ 100 kg) or 90 mg (baseline BW > 100 kg) SC on day 1, wk 4, wk 16, wk 28 and wk 40[a] At wk 28, subjects receiving ustekinumab re-randomised to continue ustekinumab or switch to ABP 654[b]	Primary: Efficacy of ABP 654 compared with Stelara (ustekinumab) Secondary: safety and immunogenicity of ABP 654 compared with Stelara (ustekinumab)	ABP 654: 281/16 Ustekinumab: 282/10	Up to 52 wks[c]	368/195 44.4 yrs (18 to 75 yrs)	• Men and women ≥ 18 years and ≤ 75 years of age • Stable moderate to severe Ps for at least 6 months • Baseline score of PASI ≥ 12, involvement of ≥ 10% BSA, and sPGA ≥ 3 at screening and at baseline	Primary: PASI percent improvement from baseline to wk 12 Secondary: • PASI percent improvement at other timepoints • PASI 75 response throughout the study • PASI 100 response throughout the study • sPGA responses (0/1) at wk 12 and wk 52 • BSA change from baseline at wk 12 and wk 52

BSA = body surface area; BW = body weight; EU = European Union; M/F = male/female; PASI = Psoriasis Area and Severity Index; PASI 50/75 = ≥ 50%/75% improvement in PASI; Ps = plaque psoriasis; SC = subcutaneous; sPGA = Static Physician's Global Assessment; wk(s) = week(s); yrs = years

[a] For subjects with PASI 50 response or better but less than PASI 75 response at week 28, dosing frequency may have been increased to every 8 weeks based on investigator discretion (ie, dosing at wks 28, 36, and 44).

[b] For subjects with a PASI 75 response or better and for subjects with PASI 50 response or better but less than PASI 75 response at week 28 who did not dose intensify [c] Does not include screening period.